



SUPERNUMERARY NIPPLES

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Supernumerary nipples, also known as accessory nipples, are a relatively common congenital anomaly where individuals have one or more additional nipples. These extra nipples are present at birth and often appear as small, brown to pink papules or areola-like structures along the milk line, which runs from the axilla to the groin. They may be solitary or multiple and can be present both unilaterally or bilaterally. If these nipples have associated glandular tissue, they may enlarge during puberty, become tender premenstrually, and undergo lactation post-partum. Additionally, supernumerary nipples can develop hair at puberty. While supernumerary nipples are generally harmless and do not require treatment solely based on their presence, medical evaluation may be necessary if they are associated with underlying conditions or cause cosmetic concerns. Differential diagnosis includes moles, cysts, or skin tags, which may mimic the appearance of supernumerary nipples.

Diagnosing supernumerary nipples involves a physical examination and can include further investigations, such as ultrasound or mammography, which may be recommended if there is concern for underlying breast tissue or associated abnormalities. Biopsy provides definitive diagnosis as histology resembles that of a normal nipple: epidermal thickening, mild papillomatosis, basal hyperpigmentation, pilosebaceous structures, smooth muscle, breast ducts, and possibly breast tissue.

In most cases, treatment is not necessary unless the extra nipples cause discomfort or cosmetic distress. Surgical removal of supernumerary nipples can be an option for those who wish to eliminate them for aesthetic reasons. However, consulting with a healthcare professional to determine the appropriate course of action based on individual circumstances is essential.

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